



Preterm birth?

Here's your guide





Message from @birth-hood

Hi,

I'm Leanne, mum of two, (and a cocker spaniel, Ron) Doula, Perinatal Yoga teacher and Birth Trauma Resolution practitioner, and obviously a hypnobirthing nerd!

When you become pregnant, for the first, or the fifth time, your mindset changes instantly, as well as being excited, it can be overwhelming and full of uncertainty. Especially if your baby decides to come early- this is known as 'Preterm'.

It can be daunting if you're told your baby needs to be born early, similarly if you have a spontaneous preterm labour, but know you and your baby's safety and wellbeing is always paramount.

I adore teaching birth preparation, supporting you and your partner as you adjust to this new phase of your journey, so I have created this little guide to help to answer any questions and give you a few tools to help you prepare for your preterm journey.

If you have any questions, please don't hesitate to contact me.

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Leanne xx

What is preterm birth?

Preterm birth (PTB) is a fairly common complication of pregnancy, comprising around 8% of births in England and Wales. We consider any baby born before 37 weeks to be preterm (NHS 2019). So you're not alone.

What do I need to do?

Call your midwife or maternity unit if you're less than 37 weeks pregnant and you have:

- regular contractions or tightenings
- period-type pains
- a gush or trickle of fluid from your vagina – this could be your waters breaking
- backache that's not usual for you

The midwife or hospital will offer checks, tests and monitoring to find out whether:

- your waters have broken
- you're in labour
- you have an infection

These may include a vaginal examination, blood test, urine test and cardiotocography to record contractions and the baby's heartbeat.

You can accept or decline any of these.

In the next few pages I will share some potential reasons for preterm birth, it is important to note that sometimes babies do just come when they're ready!



Why can it happen?

Most of the time, premature births happen spontaneously (naturally) and doctors will not know why.

Sometimes, a premature birth is planned because it's safer for mum/parent and/or the baby. This could be because either have a health condition, such as severe pre-eclampsia, that means it is safer not to continue the pregnancy.

There are some things that increase the risk of going into labour early, so some people may be told that they have a higher risk of giving birth early.

Previous premature birth

If you've given birth early before, you are more likely to give birth early again. The more premature deliveries you have had, and the earlier your babies were born, the higher this risk becomes, but there is no guarantee.

Previous late miscarriage

The causes of late miscarriage and premature birth can be the same. If you had a late miscarriage before then you may have a higher chance of having a late miscarriage or a premature birth in a next pregnancy. It doesn't mean that it will happen though. The more the doctors know about the reasons for the previous experience, the more they will be able to make sure it doesn't happen again.

Low PAPP-A levels

Pregnancy Associated Plasma Protein-A (PAPP-A) is a hormone that is made by the placenta in pregnancy. It is measured as part of the antenatal screening tests you are offered during pregnancy.

Low levels of PAPP-A may mean that the placenta (the organ that helps the baby grow and develop) may not work as well. There may be an increased chance of premature birth or pre-eclampsia.

If you are diagnosed with low levels of PAPP-A you'll likely be offered extra scans to monitor your baby's growth. It's also important to get to know your baby's movements and contact your midwife or maternity unit immediately if you think your baby's movements have slowed down, stopped or changed.

Placenta praevia

Sometimes the placenta attaches low down in the uterus and may cover part of or all of the cervix. In most cases, the placenta moves upwards and out of the way with the uterus as it grows, and as pregnancy continues, but sometimes it doesn't.

This condition is known as low-lying placenta if the placenta is less than 20 mm from the cervix or as placenta praevia if the placenta covers the cervix. If you have placenta praevia, you are at a higher risk of giving birth early.

Why can it happen?

Polyhydramnios (too much amniotic fluid)

Amniotic fluid is the fluid that surrounds your baby in the uterus. Polyhydramnios is where there is too much amniotic fluid around the baby during pregnancy. This slightly increases the risk of pregnancy complications, including premature birth.

We don't always know why too much fluid builds up in pregnancy, but it can be caused by things like:

- being pregnant with more than 1 baby.
- diabetes and gestational diabetes
- a blockage in the baby's gut (gut atresia)
- an infection during pregnancy
- the baby's blood cells being attacked by the mother's blood cells (rhesus disease)
- your baby having a genetic condition.

Too much amniotic fluid is normally spotted during a check-up in the later stages of pregnancy. It is not usually a sign of anything serious, but you may have some extra check-ups during pregnancy. Try not to worry though as most babies whose mothers have polyhydramnios will be healthy.

PPROM or waters breaking early

If your waters broke early in a previous pregnancy then there is a small chance that this will happen again. PPRM can cause late miscarriage or premature birth.

Uterine abnormalities

A uterine abnormality is a uterus that formed in an unusual way before birth. Depending on the shape of the womb, there may be a higher risk of premature birth.

Most women are unaware that they may have an abnormally shaped womb when they get pregnant. It is usually discovered during investigations for infertility or miscarriage.

Bleeding after 24 weeks

Bleeding after the first trimester can be a sign that there is a problem with the placenta, such as a low-lying placenta or placental abruption, which can both cause premature birth. Any bleeding in pregnancy should be investigated, even if you are not experiencing any other symptoms. If you are more than 12 weeks pregnant and have bleeding, go to your local A&E or contact the hospital maternity unit so you can be checked, just in case.

Problem with your cervix (a weak cervix)

Sometimes the neck of the womb (cervix) shortens during pregnancy which may indicate that you are at risk of preterm birth. Doctors do not always know why this happens, but it can happen if the cervix has been damaged. For example, from a tear in childbirth or treatment for an abnormal cervical screening, such as large loop excision of the transformation zone (LLETZ) or a cone biopsy.

When this happens, the cervix is called weak or incompetent (?! HATE), which means it could open or shorten early. This could lead to premature labour. If scans show that there is a problem with your cervix, you may be offered a cervical stitch (also known as a cerclage or cervical suture) or hormone treatment (such as progesterone) to stop your baby being born too soon.

Why can it happen?

Multiple pregnancy (twins, triplets or more)

Twins and triplets are often born prematurely. More than 50% of twins and almost all triplets are born before 37 weeks (Though this does not distinguish between planned or spontaneous labour). Even if you don't go into premature labour, you may be advised to give birth before your due date to reduce the risk of complications.

Some of the treatments available to delay premature birth, such as progesterone pessaries, a cervical stitch or Arabin Pessary are not routinely offered to women carrying multiple babies.

You may be offered a medication called corticosteroids during your pregnancy to help your baby's lungs develop more quickly. This can reduce the risk of complications associated with giving birth prematurely.

Twin-to-twin transfusion syndrome (TTTS)

Twins that share a placenta (monochorionic pregnancies) also share the same blood supply. In around 15 in 100 monochorionic twin pregnancies, the blood flow may be unbalanced. This means that 1 baby receives too little blood and has a low blood pressure, while the other baby receives too much blood and has a high blood pressure. This is called twin-to-twin transfusion syndrome (TTTS).

TTTS can sometimes lead to a premature birth. If you are carrying twins, you will be monitored with frequent scans for signs of TTTS.

The Twins Trust [has more information about TTTS](#), including symptoms to look out for.

Type 1 and type 2 diabetes

Premature birth is one of the most common problems associated with diabetes and pregnancy. But managing your blood sugars well before and during your pregnancy, will help to reduce the risk.

Gestational diabetes

Women with gestational diabetes are sometimes 'recommended' early birth by induction or caesarean section if the healthcare team are worried about the baby or mother's health, and risk of complications.

Some studies have also shown that some women with gestational diabetes are at higher risk of spontaneous premature birth.

But try not to worry. Most women with gestational diabetes have healthy pregnancies and healthy babies, especially if their condition is diagnosed and managed well during pregnancy.

Pre-eclampsia

Pre-eclampsia is a condition that affects some pregnant women. It is a combination of raised blood pressure (hypertension) and protein in your urine (proteinuria). Pre-eclampsia usually develops after 20 weeks of pregnancy or soon after the baby is delivered. Pre-eclampsia can range from mild to severe. If the pre-eclampsia is severe, it may be recommended that the baby is delivered early.

Why can it happen?

Waters breaking early (PPROM)

Preterm prelabour rupture of membranes (PPROM) is when your waters break prior to labour before 37 weeks of pregnancy.

About 50% of women with PPROM will go into labour within the first week after their waters break. The further along you are in your pregnancy the more likely you are to go into labour within 1 week of your waters breaking. PPROM is associated with 3-4 out of every 10 premature births.

Fetal growth restriction (FGR)

Fetal growth restriction (FGR) is a condition where a baby is smaller than expected or when a baby's growth slows or stops during pregnancy. It is also called intrauterine growth restriction (IUGR).

If your baby has FGR, there is an increased risk of complications in pregnancy. Your healthcare team will monitor the baby's growth and wellbeing closely and you may be advised to give birth earlier than your due date.

Intrahepatic cholestasis of pregnancy (ICP)

Intrahepatic cholestasis of pregnancy (ICP), also known as obstetric cholestasis, is a liver disorder that can affect you during pregnancy.

Normally, bile acids flow from your liver to your gut to help you digest food. In ICP, the bile acids build up in your body instead. ICP has no known cure except delivery of the baby. ICP increases the risk of premature birth. Most of these preterm deliveries are started medically (induction or caesarean), rather than happening naturally to reduce the risk of complications.

Infection in the uterus

Intrauterine infection is infection within the womb. Infections that could go up to the womb include E-Coli and Group B Streptococcus (GBS), bacterial vaginosis, chlamydia, trichomona, gonorrhoea, syphilis and HIV. Intrauterine infection can cause premature birth.

Urinary tract infections

UTIs can cause premature birth if they are not treated. It is not an emergency if you get a UTI in pregnancy, but it should be treated as soon as possible.

Placental abruption

Placental abruption is when the placenta starts to come away from the inside of the uterine wall. It can cause stomach pain, bleeding from the vagina and contractions. This can increase the risk of premature birth.

If it is serious, or you are close to your due date, the baby may need to be born immediately (usually by caesarean). But if you are still early on in your pregnancy and the abruption is minor, you may just be monitored closely, usually in hospital.



What are the risks to my baby of being born early?

Babies born before full term (before 37 weeks) are vulnerable to problems associated with being born premature. The earlier in the pregnancy a baby is born, the more vulnerable they are.

Not all of these babies will need to be cared for in a neonatal unit. Many babies born prematurely will be born late preterm (at 34-36 weeks of pregnancy), and some of these may not need specialist care on a neonatal unit.

For every 1,000 babies who need neonatal care in the UK, we estimate that:

- 25 are extremely preterm (being before 28 weeks of pregnancy)
- 51 are very preterm (being born between 28 weeks and less than 32 weeks of pregnancy)
- 283 are moderately preterm (being born between 32 weeks and less than 37 weeks of pregnancy)
- 641 are born at full term (being born at 37 weeks or more of pregnancy)

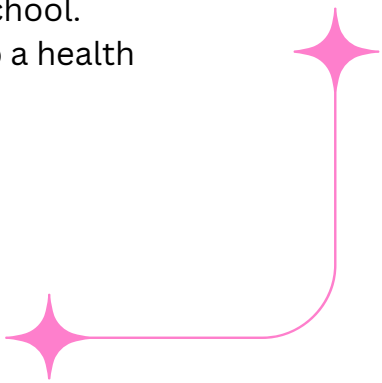
It's possible for a baby to survive if born at around 24 weeks of pregnancy.

Babies born this early need special care in a hospital with specialist facilities for premature babies. This is called a neonatal unit. They may have health and development problems because they have not fully developed in the womb.

If your baby is likely to be delivered early, you should be admitted to a hospital with a neonatal unit.

Not all hospitals have facilities for the care of very premature babies, so it may be necessary to transfer you and your baby to another unit.

It is important to remember that every baby is different and will develop differently. As some babies who were born prematurely develop into children and adults, they may suffer from conditions such as behavioural difficulties, long-term health problems or cerebral palsy. Some research has also found that the earlier a baby is born the higher their risk of having special educational needs at school. If you are worried about your baby's development please speak to a health professional.



What treatment could I receive?

Planned premature labour

In some cases, pre-term labour is planned and induced because it's safer for the baby to be born sooner rather than later.

This could be because of a health condition in the mother (such as pre-eclampsia) or in the baby. Your midwife and doctor will discuss with you the benefits and risks of continuing with the pregnancy versus your baby being born premature.

You can still make a birth plan (and I strongly suggest you do!) and discuss your wishes with your birth partner, midwife and doctor.

If your waters have broken

If your waters have broken (called preterm pre-labour rupture of membranes, P-PROM), there's an increased risk of infection for you and your baby. You may be offered:

- antibiotics to take
- tests for infection

P-PROM does not definitely mean you're going into labour but you may be advised to stay in hospital for a few days.

When you go home, you'll be advised to call your midwife or maternity unit immediately if:

- your temperature is raised
- you feel hot and shivery
- any fluid coming from your vagina is greenish or smelly
- you bleed from your vagina
- you have pain in your tummy or back
- you have contractions
- your baby's movements slow down or stop or there is a change to their usual pattern of movements

If your pre-term labour is planned or unplanned or if your waters have broken prematurely, you may be offered steroid injections.

You'll be offered immediate induction or a caesarean section if:

- you have P-PROM between weeks 34 and 37 of pregnancy, and a test has found group B strep bacteria in your pee or vagina

If your waters have not broken, your midwife or doctor should discuss with you the symptoms of pre-term labour and offer checks to see if you're in labour.

These checks can include asking you about your medical and pregnancy history, and about possible labour signs, such as:

- contractions – how long, how strong and how far apart they are
- any pain
- any fluid coming from your vagina
- You may be offered a vaginal examination, and your pulse, blood pressure and temperature may also be checked.

Your midwife or doctor will also check your baby. They'll probably feel your bump to find out the baby's position and how far into your pelvis the baby's head is.

They should also ask about your baby's movements in the last 24 hours. If they do not ask, tell them about the baby's movements.

If you're in premature labour the midwife or doctor may offer:

- medicine to try to slow down or stop your labour
- steroid injections, which can help your baby's lungs
- Slowing down labour or stopping it is not appropriate in all circumstances – your midwife or doctor can discuss your situation with you.

They will consider:

- how many weeks pregnant you are
- whether it might be safer for the baby to be born – for example, if you have an infection or you're bleeding
- local neonatal (newborn) care facilities and whether you might need to be moved to another hospital
- your wishes

You may be offered a course of steroid injections to help your baby's lungs get ready for breathing if they're born prematurely.

Steroids may not be offered after 36 weeks as your baby's lungs are likely to be ready for breathing on their own.

Post-birth interventions?

Low Blood Sugars (Hypoglycaemia)

Your baby will be monitored for at least the first 48-72 hours of life to ensure their blood sugars are within a normal range. The usual threshold level is around 2.6 mmol/L. If the blood sugar falls below this level, you will have discussions around feeding and see how you can be supported further. Your baby will have three pre-feed blood sugar tests in a row, then at 24, 36, 48 and 72 hours of age.

Feed your baby as they show feeding cues and with gaps of no longer than three hours in between feeds.

Jaundice

Jaundice is the yellow colouration of the skin, eyes and mucous membranes (NHS, 2018).

It's very common – around 80% of preterm infants and 60% of term infants (NICE 2018) will become jaundiced. Not all infants who have jaundice will need treatment for it. If your HCP is concerned about your baby looking jaundiced, they will check their levels with a heel prick blood test. You may notice your baby becoming increasingly yellow, lethargic and they may be reluctant to feed. If your baby does need treatment, they can usually have this on the postnatal ward and they will receive a treatment called phototherapy (NHS, 2018). They measure the jaundice levels regularly and we will let you know when your baby can stop phototherapy.

Low temperature (Hypothermia)

Preterm babies are more likely to have a lower temperature than a term baby as they are usually smaller and have less fat. Your HCP will likely recommend that you have as much skin to skin contact as possible as this helps to regulate your baby's temperature, among other benefits such as bonding and for milk production (UNICEF 2019). While in the cot make sure the baby is appropriately dressed (usually one more layer than adults, however depending on the temperature of the ward you may need extra/fewer layers). Soon after birth they might suggest putting a hat on your baby, but it is not recommended that your baby sleeps with a hat on at home (Lullaby trust, 2020). Your HCP will check your baby's temperature frequently, as being cold can lead to low blood sugars, breathing problems and weight loss.

Weight loss

your HCP will weigh your baby on day 3 and again on day 5 if you are still in hospital, otherwise this will be done in the community. They expect newborns to lose weight in their first week of life (NHS, 2020), but want to make sure it's not more than a 10% loss. If your baby has lost more than 10% they will implement an individualised feeding plan to help your baby gain weight. They will weigh your baby again prior to going home to ensure they are gaining weight appropriately.